

Institutional Investor Opportunity Summary — True Care System, LLC

Production Healthcare SaaS Platform | Home Care • I/DD • Behavioral Health • Community-Based Services | August 2026

COMPANY

True Care System, LLC
Healthcare SaaS — Multi-Tenant
Cloud Platform
Pennsylvania, USA · Founded 2023

ASSET PROFILE

Product	Live in Production
Development	Oct 2023–Jul 2026
Validation	34 Months
Sandata EVV	Prod. Certified
Security	HIPAA Framework
Architecture	Multi-Tenant Cloud
Mobile	Native iOS + Android

HISTORICAL INVESTMENT

Founder Operational	\$1,227,000
Development Expense	\$220,000
Total to Date	\$1,447,000
Institutional Capital	\$0 — clean cap table

TRANSACTION FRAMEWORK

Structure	Strategic Equity
Ownership Available	Up to 49%
Governance	Negotiated Jointly
Founders	Remain in Leadership

UNIT ECONOMICS

Pricing	\$25 / individual / mo
Annual Escalator	~5%
Gross Margin Target	80%
2027 Target ARR	~\$1.7M
2031 EBITDA Margin	~43%
Break-Even ARR	~\$1.38M

CONTACT

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Confidential — Qualified
Institutional Investors

\$760B+

COMBINED TARGET
MARKETS

~\$12.5M

2031 TARGET ARR

~43%

2031 EBITDA MARGIN
TARGET

≤49%

OWNERSHIP AVAILABLE

Executive Summary

True Care System, LLC is a **production-stage, multi-tenant healthcare SaaS platform** serving Home Care, I/DD, Behavioral Health, and Community-Based Service providers. Developed and operationally validated over 34 months (October 2023 – July 2026) with production deployment since 2024, the platform unifies workforce management, Electronic Visit Verification (EVV), scheduling, billing, payroll, compliance, reporting, and mobile operations in a single enterprise environment — operating daily inside an active Pennsylvania provider. The founders have funded the Company entirely — **\$1,447,000 in documented historical investment, zero external capital** — and are seeking a strategic institutional partner for **up to 49% ownership** to fund commercialization and disciplined multi-state expansion. The result is a clean-cap-table entry into a de-risked, revenue-ready recurring-revenue asset.

Investment Thesis

- **Technology execution risk substantially retired.** The platform is production-deployed with real regulated workloads — clinical, billing, payroll, EVV, and compliance — validated by an operating healthcare provider, not pilots or prototypes.
- **Regulatory moat in place.** Sandata Production Certification (registered EVV vendor organization) and a HIPAA-oriented security and audit framework are prerequisites for Medicaid-funded markets and materially raise barriers for uncertified entrants.
- **Capital deploys into growth, not product creation.** Use of proceeds targets sales, implementation capacity, marketing, and expansion — the core platform is complete and paid for.
- **Attractive SaaS economics.** \$25 per active individual per month (industry average ~\$35; internal reference price \$30) with a ~5% annual escalator, an 80% gross-margin planning target, and projected EBITDA margin expansion from ~16% (2027) to ~43% (2031).
- **Aligned management.** Founders retain majority ownership, remain in leadership post-transaction, and will structure governance collaboratively with the incoming partner.

Market Opportunity

The platform addresses two structurally expanding U.S. markets: **home care (\$150B+, 2024 est.)** and **behavioral health (\$610B+, 2024 est.)**, each growing 8–10%+ annually on demographic aging, CMS and state Medicaid funding expansion, and the Mental Health Access Improvement Act's widening of reimbursable licensed professionals (LPC, LCSW, LMFT). Regulatory pressure — EVV mandates, HIPAA obligations, payer documentation — is pushing providers from fragmented point tools toward certified, integrated platforms. In the initial Pennsylvania market alone, the DHS provider directory lists **9,426 licensed ODP providers** across 67 counties — one program within a substantially larger total provider universe.



Financial Projections, Transaction Structure & Path to Liquidity

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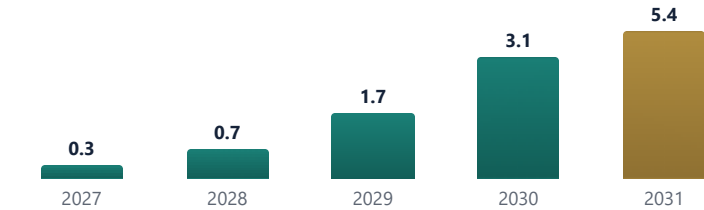
Five-Year Financial Projections (2027–2031)

Commercialization begins in Pennsylvania in a phased, capital-disciplined rollout — validate onboarding and pricing in-state, expand to neighboring states, then scale multi-state where product fit and regulatory readiness are demonstrated. Projections assume growth from 22 providers (5,720 active individuals) in 2027 to 90 providers (34,200 individuals) in 2031 — a 39% CAGR.

YEAR	REVENUE	GROSS PROFIT	EBITDA	MARGIN
2027	\$1.72M	\$1.37M	\$0.27M	15.9%
2028	\$3.11M	\$2.48M	\$0.74M	23.8%
2029	\$5.29M	\$4.23M	\$1.66M	31.3%
2030	\$8.27M	\$6.61M	\$3.13M	37.9%
2031	\$12.47M	\$9.98M	\$5.36M	43.0%
5-Yr Revenue	~\$30.9M	80% gross-margin planning target		

KEY PLANNING METRICS	
Break-even ARR (2027 cost base)	~\$1.38M (~4,600 individuals / ~18 providers)
Indicative operating cash flow (2031)	~\$5.2M
Customer retention target	≥95%
System availability target	≥99.5%

Projected EBITDA (\$M) & Margin, 2027–2031



EBITDA margin expands 15.9% → 43.0% as recurring revenue scales over a largely fixed platform cost base.

Operating leverage: a multi-tenant platform already built means each incremental provider adds recurring revenue at an 80% target gross margin, while operating expenses grow at less than half the rate of revenue in the plan (\$1.1M → \$4.6M vs. \$1.7M → \$12.5M).

Transaction Structure & Governance

The founders' initial framework makes available **up to 49% of the Company's ownership** for a qualified strategic investment, with founders retaining majority ownership and operating leadership. For a partner contributing exceptional strategic value — provider networks, multi-state operating capability, enterprise sales infrastructure, or technology resources — the founders will consider **alternative structures, including combined founder ownership below 50%**. Governance rights, board composition, valuation, and final terms are preliminary and will be established through due diligence and definitive agreements. Potential long-term liquidity pathways include strategic merger, acquisition by a healthcare technology / EVV / workforce-management platform, or private-equity growth investment.

USE OF PROCEEDS (PRELIMINARY)	
Product, Engineering & Security	25%
Sales & Provider Acquisition	20%
Marketing & Brand Development	20%
Implementation, Success & Support	15%
Cloud Infrastructure & Scalability	10%
Working Capital & Organization	10%

HISTORICAL FOUNDER INVESTMENT (100% FOUNDER-FUNDED)	
Operational contribution (9,760 documented hours)	\$1,227,000
Direct development expenses	\$220,000
Total historical investment	\$1,447,000
External / institutional capital raised	\$0 (0%)

Risk Management & Institutional Readiness

- Execution:** phased Pennsylvania-first expansion matches spend to demonstrated demand; break-even at ~18 providers in a 9,400+ provider home market limits downside on the base plan.
- Regulatory:** production Sandata EVV integration, HIPAA-oriented audit and PHI-access monitoring, and continuous compliance tracking are already operating — not roadmap items.
- Technology:** modern cloud-native stack (Next.js, NestJS, PostgreSQL, multi-tenant isolation) with CI/CD, monitoring, automated backup and disaster recovery, and an AI-ready architecture.
- Team:** Dr. Van Duong Chau (CEO, 20+ years leadership; also CEO of a Pennsylvania care provider), Thi Thuy Hang Tran, MHA (COO, 18+ years operations), Tu Chau (Project Director) — with a phased hiring plan to 19 employees by 2031.

Next step: qualified institutional investors are invited to begin a strategic-fit review and request the Confidential Investment Memorandum and due-diligence materials — contact **Dr. Van Duong Chau, CEO** at **admin@truecaresystem.com**.